

PDF Skin Health & Wound Management — Monthly Sample Audit

QAPI Quality Improvement Audit Tool · Companion to the PAWSIC Center Self-Assessment

QUALITY IMPROVEMENT TOOL — PROTECTED & CONFIDENTIAL

This is a Quality Improvement (QI) tool prepared for and used exclusively by the facility's Quality Assessment and Assurance (QAA) / QAPI Committee in accordance with 42 CFR § 483.75(g) and (h). It is privileged, confidential, and protected from disclosure under federal and applicable state law, except for the limited purpose of demonstrating compliance with QAPI regulations.

Purpose & Scope. A focused monthly QAPI audit of a small resident sample to validate that the systems measured in the Center Self-Assessment are working at the bedside. Pair each finding with the matching self-assessment section and feed gaps into your SMART-goal action plan.

Sample. 5 residents (recommended: one per unit/floor). Prioritize in-house acquired, full-thickness, declining, or non-healing wounds of any etiology, and any resident on Enhanced Barrier Precautions. Record resident initials and room number.

Method. Combine record review, interview, and direct observation ("Ask, Observe, Validate"). Score every element for each resident using the key below.

How to Score — Keys

1	Met — present and accurate	0	Not met — absent or incorrectly met	NA	Not applicable
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Scoring note: Tally each resident's "1" responses and divide by the number of applicable (non-NA) items to get a compliance %. NA items are excluded from that resident's denominator.

F-Tag Quick Key (CMS SOM Appendix PP):

F686	Treatment / services to prevent & heal pressure ulcers and wounds	F880	Infection prevention & control (incl. Enhanced Barrier Precautions)
F684	Quality of care — each resident receives necessary care/services	F656	Comprehensive person-centered care plan
F641	MDS assessment accuracy	F603	Freedom from involuntary seclusion (avoid over-isolation)

Monthly Audit Grid — 5-Resident Sample

Score each element 1 / 0 / NA per resident. Shaded bands group elements by domain; the F880 band (amber) covers Infection Control & Enhanced Barrier Precautions.

Audit Element to Verify	F-Tag	Res 1	Res 2	Res 3	Res 4	Res 5
I. Resident Selection & Identifiers						
Resident initials & room #	—					
Unit / floor	—					
Wound type / stage / status	—					
IHA or POA · wound origin date	—					
II. Paper Trail — Assessment & Documentation						
Risk Assessment: Captures all risk factors; completed on admission/readmission, weekly x4, quarterly, and change in condition; risks matched to interventions.	F686 / F656					
Skin Checks: Completed on new admit, weekly, change in condition, and discharge.	F686					
CEP Completed: Critical Element Pathway tells the “full story” for in-house / declining wounds.	F686 / F684					
MDS Accuracy: Correct stage and count; matches the wound log.	F641 / F686					
III. Bedside-Chart Alignment Review						
Wound Assessment Accuracy: Documentation (and image if applicable) matches the observed wound.	F686					
Treatment: Performed per current MD order; misses explained and communicated.	F686					
Interventions Present: Listed interventions are physically present / on / used by the resident per the care plan.	F686 / F684					
IV. Infection Control & Enhanced Barrier Precautions (F880)						
EBP Applied to Wound Care: Gown + gloves used for high-contact wound care for any resident with a chronic wound (PI any stage, DFU, unhealed surgical, venous stasis) or indwelling device — regardless of MDRO status (observe).	F880					
Correct Precaution Triage: Drainage contained by intact dressing = EBP (not room-restricted); uncontrollable drainage = Contact Precautions. Resident is not over-isolated.	F880 / F603					
Hand Hygiene & PPE Sequence: Correct don (HH → gown → gloves) and doff (gloves → gown → HH); glove change + HH when moving from a contaminated to a clean site (observe).	F880					
Aseptic Technique & Covering: Clean gloves never contact the wound bed; key parts protected; the wound is covered at all times outside of wound care/dressing change (observe).	F880 / F686					
Supplies, Environment & Waste: Supplies gathered before entry; single-use items not reused; no supply/dressing cart in the room; surface disinfected with EPA product; soiled dressings in in-room waste.	F880					

Audit Element to Verify	F-Tag	Res 1	Res 2	Res 3	Res 4	Res 5
EBP Readiness & Competency: Signage posted; gown/gloves available at the point of care; staff EBP competency (return demo) documented.	F880 / F684					
V. Care Plan Alignment						
Wound Coverage: Care plan reflects every wound type the resident has.	F656 / F686					
Risk–Intervention Match: Interventions match the documented risk factors and wound types.	F656					
VI. ADL & Support Services						
ADL Observation: Safe skin handling and routine skin inspection during care (observe).	F684					
Skin Care & Continence: Creams to legs/arms 2x/day; continence managed and barrier applied as indicated.	F686 / F684					
RESIDENT COMPLIANCE SCORE (count of “1” ÷ applicable items)		___ / ___	___ / ___	___ / ___	___ / ___	___ / ___

Quantitative Analysis for the QAPI Committee

Summarize the grid above. Compliance % = (count of “1” ÷ applicable items) × 100 for each resident.

Metric	Res 1	Res 2	Res 3	Res 4	Res 5
Applicable items (denominator)	—	—	—	—	—
Items met (“1”)	—	—	—	—	—
Compliance %	— %	— %	— %	— %	— %

Thematic Findings (read down each column)

Systemic Strengths — elements scoring “1” across all 5 residents:

Systemic Gaps — elements scoring “0” for 3 or more residents:

Infection Control / EBP (F880) observations — any precaution, PPE-sequence, or technique gaps:

Root Cause Worksheet — the “Why”

If a theme emerged (e.g., 3/5 residents missing an intervention, or EBP not in use), check the likely cause(s).

- ☐ Process / system: e.g., equipment (low-air-loss surfaces, boots) or EBP supplies (gowns/gloves at point of care) not available.
- ☐ Communication: e.g., wound-nurse recommendations not transcribed to the care plan; EBP status not flagged on signage / EHR.
- ☐ Clinical knowledge: e.g., staff cannot distinguish wound types or PI Stage 4 vs. Unstageable, or EBP vs. Contact Precautions.
- ☐ Documentation: e.g., double-charting, or the CEP for IHA skipped during high-acuity shifts.
- ☐ Technique: e.g., incorrect don/doff sequence, clean gloves contacting the wound bed, wound left open to air.

Other:

Findings Summary & Follow-Up

Summarize this month’s audit, then log each action and its re-audit outcome. Carry unresolved gaps into the Center Self-Assessment SMART-goal action plan.

Audit month: _____ Unit(s) audited: _____ Auditor: _____

Summary of Findings

Follow-Up & Action Tracker

Date	Finding / Gap (F-Tag)	Action & Owner	Target Date	Re-Audit Result	Status
					Open / In Progress / Closed
					Open / In Progress / Closed
					Open / In Progress / Closed
					Open / In Progress / Closed
					Open / In Progress / Closed

Quarterly QAPI Summary — Wound Care & Skin Integrity

Aggregate monthly audits into a high-level executive summary for the Medical Director, Administrator, or survey request — evidence of active oversight and continuous improvement.

Facility: _____ Review period: Q1 Q2 Q3 Q4 Year: _____

Total residents audited this quarter: _____ (average 5–10 per month)

1. Executive Performance Summary

Key Performance Indicator (KPI)	Goal	Actual	Pass / Fail
Interventions & treatments accurate and complete	95%	_____ %	
Documentation complete (wound assessment, skin checks, risk assessment)	100%	_____ %	
Infection Control & Enhanced Barrier Precautions compliance (F880)	100%	_____ %	
Wound Log – Care Plan – MDS alignment	95%	_____ %	

2. Trend Analysis (the “full story”)

Positive trends identified:

Persistent barriers:

Of the declining / non-healing wounds audited, how many were successfully stabilized? _____

3. Root Cause Analysis & Actions Taken

Primary issue identified this quarter:

Corrective actions implemented:

4. Quality Improvement Goals for Next Quarter (use SMART format)

Example (F880): “By [date, within 60 days], 100% of nurses and CNAs providing wound care will pass an Enhanced Barrier Precautions return-demonstration, raising observed EBP compliance from [baseline]% to ≥ 90% on the monthly audit.”

Goal 1:

Goal 2:

Goal 3:

Committee Sign-Off

Medical Director: _____ Date: _____

Administrator: _____ Date: _____

DON / QAPI Chair: _____ Date: _____